

St. Mary's Medical Center

DISCHARGE SUMMARY

Patient: Margaret Thompson

DOB: 03/15/1958 (Age 68)

MRN: MRN-2024-0847

Admission Date: 01/05/2026

Discharge Date: 01/09/2026

Attending Physician: Dr. James Chen, MD

ADMISSION DIAGNOSIS:

1. Acute exacerbation of chronic heart failure (I50.9)
2. Type 2 Diabetes Mellitus (E11.65)
3. Hypertension (I10)

HISTORY OF PRESENT ILLNESS:

Ms. Thompson is a 68-year-old female with a past medical history significant for CHF (EF 35%), type 2 diabetes, and hypertension who presented to the ED with 3 days of progressive dyspnea, lower extremity edema, and weight gain of 8 lbs over the past week. Patient reports she was unable to maintain her usual low-sodium diet during the holiday season.

DISCHARGE MEDICATIONS:

1. Lisinopril 20 mg PO daily
2. Furosemide 40 mg PO daily
3. Metformin 1000 mg PO BID
4. Carvedilol 12.5 mg PO BID
5. Aspirin 81 mg PO daily
6. Spironolactone 25 mg PO daily

ALLERGIES: Penicillin (rash), Sulfa drugs (hives)

LAB VALUES AT DISCHARGE:

- BNP: 850 pg/mL (elevated, was 1200 on admission)
- HbA1c: 8.2% (elevated)
- Creatinine: 1.3 mg/dL (mildly elevated)
- Potassium: 4.8 mEq/L (normal)
- Sodium: 138 mEq/L (normal)
- Hemoglobin: 11.2 g/dL (mildly low)

VITAL SIGNS AT DISCHARGE:

- Blood Pressure: 128/78 mmHg
- Heart Rate: 72 bpm

- Respiratory Rate: 18 breaths/min
- O2 Saturation: 96% on room air
- Weight: 162 lbs (down 6 lbs from admission)

PROCEDURES:

- Echocardiogram (01/06): EF 35%, moderate MR, elevated PASP
- Coronary angiography (01/07): No significant CAD

CONDITION AT DISCHARGE:

Stable. Symptoms improved with IV furosemide and fluid restriction. Patient ambulatory, tolerating regular diet.

FOLLOW-UP:

- Cardiology follow-up in 2 weeks with Dr. Chen
- Primary care in 1 week
- Repeat BNP in 1 week
- Daily weight monitoring
- Low sodium diet reinforcement
- Diabetic education referral

DISPOSITION:

Discharged to home with home health services.